



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : فاطمه عبدالسلام جوده حسانين

العنوان : ابو النمرس

الحي / المدينة : ابو النمرس

المحافظة : الجيزة

تاريخ تسجيل الإستمارة : 03-10-2026

رقم البطاقة : 26902172200763 رقم التليفون : 01555148881

العمر (تاريخ الميلاد) : 17-02-1969 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: فاطمة عبدالسلام جوده حسانين National ID: 26902172200763	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

٢٠٢٣/٠٢ ٢٦٩٠٢١٧٢٢٠٠٧٦٣

عامله خدمات معنوقه
مستشفى الاطفال العامى التخصصى
أنشى مسلمة
سعيد سليمان على عياده
البطاقة سنارية حتى ٢٠٢٠/٠٢/١٩





جمهورية مصر العربية
بطاقة تحقيق الشخصية

فاطمه
عبد السلام جوده حسنين
قرية منيل شبحه
مركز أبو النمرس - الجيزه



١٩٩٩/٠٧/٢٧ ٢٦٩٠٢١٧٢٢٠٠٧٦٣

JW2769751

Name:	فاطمة عبدالسلام جوده حسانين	
MR. :	3166	
Detailed Medical Report		
		السيد
تحية طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
نتشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient is a 56-year-old individual with a history of right breast cancer, staged T3 N1, who has undergone neoadjuvant chemotherapy (NACT) with a taxol–carboplatin backbone followed by a phesgo (paclitaxel 600 mg SC) regimen. Initial MDT discussion on 1 December 2025 noted a good clinical response with no palpable mass and a T2 N1 status. The patient received taxol 130 mg and carboplatin AUC 2 during week 9, followed by taxol 130 mg and carboplatin AUC 2 in week 10, and then taxol 130 mg with carboplatin AUC 2 plus phesgo 600 mg SC in week 11. Subsequent cycles continued with taxol 130 mg and carboplatin AUC 2 through week 13, and phesgo was administered in cycle 4 (week 14) with taxol 130 mg and carboplatin AUC 2. In week 15 the patient received taxol 130 mg and carboplatin AUC 2 (200 mg) with labs accepted on 7 March 2026; the same regimen was repeated in week 16 with labs accepted on 21 March 2026. Week 17 included taxol 130 mg, carboplatin AUC 2, and phesgo C5, with a new CBC on 28 March 2026 showing neutropenia. The patient’s left ventricular ejection fraction remained stable at 65 % during this period. Post-NACT, the patient underwent a stereotactic radiosurgery (SRS) procedure with complete response noted on 5 April 2026. Subsequent management included a port insertion and initiation of adjuvant endocrine therapy with aromatase inhibitor (AI) and continued phesgo. On 6 June 2026, the patient received cycle 7 of phesgo 600 mg SC, femara, and supportive therapy, with an echocardiogram on 4–5 June 2026 showing EF 67 %. Cycle 8 of phesgo was administered on 12 July 2026 with continued femara and bone-protective agents (calcemate and bone care). On 6 May 2026, the patient received cycle 9 of phesgo and arimidex, with new labs and a venous duplex performed. Cycle 10 of phesgo and arimidex was given on 16 August 2026, with a venous duplex showing free flow and EF 64 %. The patient’s port was removed on 8 September 2026, after which cycle 11 of phesgo and arimidex was administered, followed by cycle 12 of phesgo and arimidex. The patient has completed 12 cycles of phesgo.		
تحريراً في ,		
	الوقت :	التاريخ : 20-09-2026
	dina makram	الطبيب المعالج :
يعتمد ,		

Patient Name : الأستاذة / فاطمه عبدالسلام جوده	Request No. : 13006324
Patient No. : 26902172200763	Request Date : 06/05/2026
Path No. : 1341/26	Verification Date : 17/05/2026
Sex : Female	Financial Category : جوستاف روسي - نفقة الدولة
Age : 57 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer, Post-NACT.

Nature of Specimen: Right breast wide local excision, Sentinel lymph nodes biopsy & Extra-lymph nodes biopsy.

Gross Examination:

- * Right breast wide local excision specimen measured 08x03x03 cm, with covering skin ellipse measured 03x02 cm.
- * Serialling showed a wire guided tumor bed site measured 2x2x1 cm, located 0.3 cm from the least inferior margin.
- * Sentinel lymph nodes: Fibrofatty tissue piece measured 4x3x1 cm, entangling 6 lymph nodes, the largest measured 1.5x1x0.5 cm.
- * Extra lymph nodes: Fibrofatty tissue piece entangling 2 lymph nodes, each measured 1x1x0.5 cm.
- * Total lymph nodes; Eight lymph nodes.
- * 12 Blocks:
- 1- Medial margin., 2- Lateral margin., 3- Superior margin., 4- Inferior margin., 5- Deep margin., 6- SLNs., 7:10- Wire guided area., 11&12- Extra SLNs.
- Gross was done by; Dr. Heba El-Shafeey.

** Frozen (Intra-operative) results:

- Surgical margins: Free.
- SLNs: Six free sentinel lymph nodes (0/6).

Microscopic Examination:

- * Histological type: No residual viable remnants of the previously diagnosed invasive tumor.
- Cellularity (0%).
- * Angioinvasion: Absent.
- * Ductal carcinoma in situ: Absent.

Verified by :

Dr.Duaa Mohamed



Patient Name : الأستاذة / فاطمه عبدالسلام جوده	Request No. : 13006324
Patient No. : 26902172200763	Request Date : 06/05/2026
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Age : 57 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

- * Other findings: Extensive therapy effect.
- * Surgical margins: Free.
- * Sentinel lymph nodes: Six free sentinel lymph nodes, SLNs (0/6).
- * Extra lymph nodes: Two free lymph nodes (0/2).
- * Total nodal status: Eight free lymph nodes (0/8).
- * Histologic response to therapy: Pathological complete response to therapy.

Diagnosis:

Right breast cancer (Post-NACT), Right breast wide local excision, Sentinel lymph nodes biopsy & Extra-lymph nodes:

- No residual viable remnants of the previously diagnosed invasive tumor.
- Ductal carcinoma in situ: Absent.
- Surgical margins: Free.
- * Total nodal status: Eight free lymph nodes (0/8).
- Histologic response to therapy: Pathological complete response to therapy.
- Stage: ypT0, ypN0 (sln).

Signature

Dr. Walaa Ibrahim Ghoneim.

Verified by :

Dr.Duaa Mohamed

Printing Date : 17/05/2026 10:37:50

Page 2 of 2

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

06/01/2026

3

MDT FORM

Name:	Fatma Abdelazem Gouda Hassanien	MRN:	3156
Age:	57 Y	ID:	26902172200763
Menopausal Status:	post	Phone:	01005766514
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	-VE	Reason Of MDT:	post op

Clinical Data:

BX: RIGHT IDC G2
ER 8 PR 7 HER 3 K167 50

Sonmamimography:

Mets Workup:

History:

Initially cT2-ntRight breast cancer (Post-NACT), Right breast wide local excision, Sentinel lymph nodes biopsy & Extra-lymph nodes: - No residual viable remnants of the previously diagnosed invasive tumor. ? Ductal carcinoma in situ - Assent, * Surgical margins: free, * Total node status: Eight free lymph nodes (0-3), - Histologic response to therapy:

Recommendations:

for adj. tx:

MDT Outcome & Plan

ECRT

CONTINUE ADJUVANT PHERGO

ADJUVANT AI

R

L



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

06/01/2026

0

MDT FORM

Name:	Fatma Abdelsalam Gouda Hassenan	MRN:	3166
Age:	57 Y	ID:	26902172200753
Menopausal Sta:	post	Phone:	01005766814
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	-VE	Reason Of MDT:	post op

Clinical Data:

DX: Right IDC G2
ER: 6 **PR:** 7 **HER2:** 3 **KI67:** 50

R

L

Sonotomography:

Mets Workup:

History:

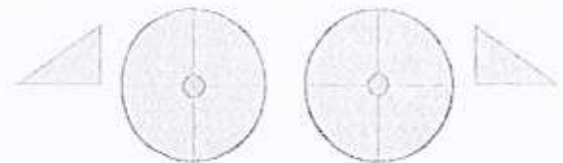
Initially cr2 n1 Right breast cancer (Post-MACT), Right breast wide local excision, Sentinel lymph nodes biopsy & Extra-lymph nodes: - No residual viable remnants of the previously diagnosed invasive tumor. ? Ductal carcinoma in situ: Absent. ? Surgical margins: Free. * Total nodal status: Eight free lymph nodes (0/8). - Histologic response to therapy: Pathological complete response to therapy. ? Stage: ypT0, ypN0 (pN).

Recommendations:

for adjuvant

MDT Outcome & Plan

PORT
 CONTINUE ADJUVANT PHESGO
 ADJUVANT AI

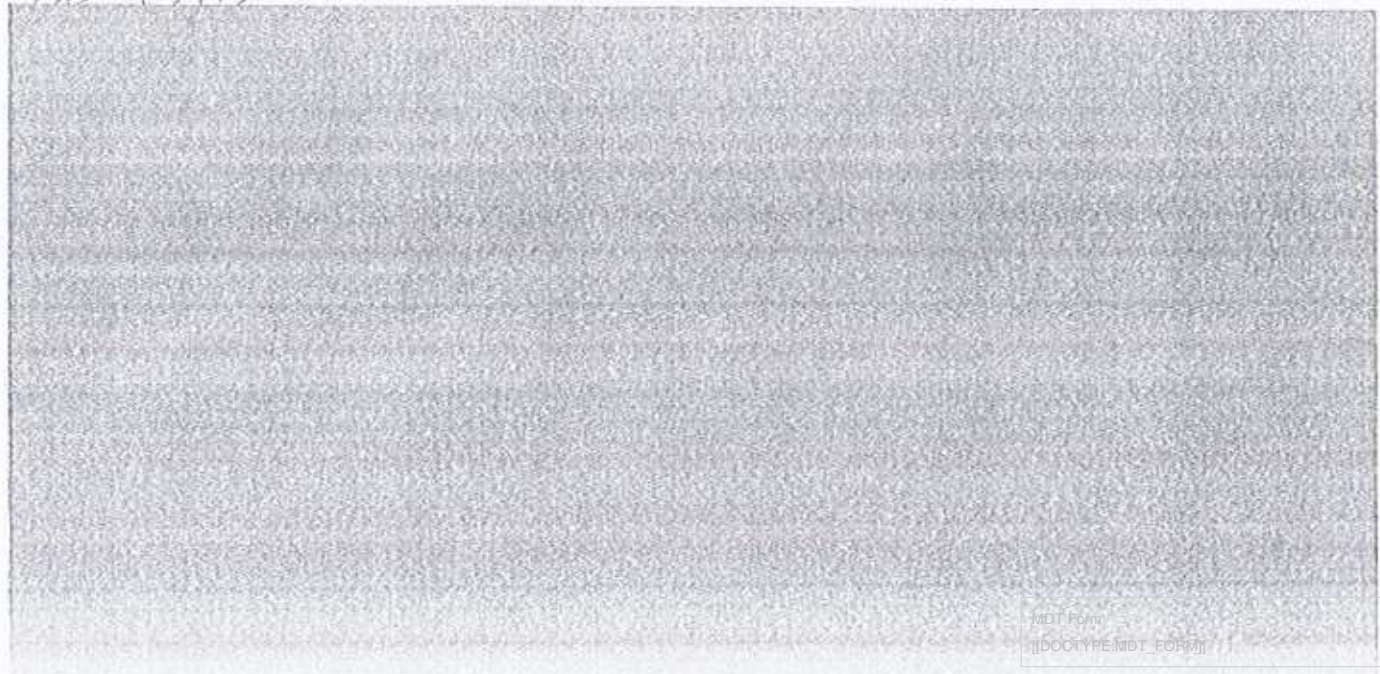


T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
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[Signature]



Patient Name : الأمثلة / فاطمة عبدالسلام جوده	Request No. : 13006324
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Sex : Female	Financial Category : جومنتف رومسي - نفقة الدولة
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Referreing Doc :	Registered by : .marwa mohamed youss

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Page 1 of 2

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Patient Name : الأستاذة / فاطمة عبدالسلام جوده	Request No. : 13006324
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Path No. : 1341/26	Verification Date : 17/05/2026
Sex : Female	Financial Category : جوماتك روسي - نفقة الدولة
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Signature

Dr. Walaa Ibrahim Ghoneim.

Verified by :

Dr. Duaa Mohamed

Printing Date : 17/05/2026 10:37:50

Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadima - Cairo, Egypt

Patient ID: 95194	Patient Name English: Fatemah Abdulsalam Godah Hassanien
Age: ٥٧ سنة ٢ شهر	2026/05/05 10:03:40 AM :Report Date

UltraSound Guided Wire localization:

Clinical summary: Pathologically proven right outer central deeply located breast cancer with clip inserted within and she completed her NACT. For pre-operative wire localization. The lesion shows complete radiological response

Technique:

- Under complete sterile conditions, 3ml of local anaesthetic were administered at the site of concern in the right breast. One 21G wire was used to localize the lesion.
- The wire is located at 9:00 10.5cm from the nipple.
- The wire is inserted transversely from the outer aspect for about 1.5cm and below the skin for 0.4cm to reach the clip with the wire is transfixing the clip . confirmed by ML view

Much obliged

Dr. Marwa Ramzy, MD, FRCR,

Patient ID: 95194	Patient Name English: Fatemah Abdelsalam Godah Hassanen
مرحلة ٦ Age:	20260505 10:03:40 AM Report Date

UltraSound Guided Wire localization:

Clinical summary: Pathologically proven right outer central deeply located breast cancer with clip inserted within and she completed her NACT. For pre-operative wire localization. The lesion shows complete radiological response

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Much obliged

Dr. Marwa Ramzy, MD, FRCR,

ECHO REPORT

NAME: فاطمة عبد الله جود

DATE: 4/5/2026

ID NUMBER: /

SEX: ♀ AGE: 58

EF	67%	55-75%	PWT	0.9	0.6-1.1cm
LVEDD	47	3.5-5.8cm	AO.R	2.7	3-3.7cm
LVESD		2-4cm	LA	3.5	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	0.8	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions

LV Wall Thickness

LV Diastolic Function

SWMA

LA and Ao Root Dimensions

Mitral Valve

Aortic Valve

Tricuspid Valve

Pulmonary Valve

Right Side and Systolic Function

Intracardiac Masses and Effusion

DR.

[Signature]

Patient's Name: fatma abd el salam gooda
Date: 2026-04-07
Referred by: Prof.Dr. CT CAP+C
Hospital: Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

(I) Chest:

- ☐ Right breast retro areolar mass is noted measuring 2.6 x 1.6 cm .
- ☐ Right enlarged axillary lymph nodes , the largest measures 13 x 6 mm with preserved fatty hilum
- ☐ Right paratracheal group lymph nodes , the largest 12 x 9mm.
- ☐ Left lower lung lobe area of consolidation intermingled with areas of ground glass attenuation .
- ☐ No sizable pulmonary masses or consolidation patches
- ☐ No pleural or pericardial collections seen.
- ☐ No gross cardiac abnormality.

(II) Abdomen and pelvis:

- ☐ The liver is enlarged in size showing homogenous CT density , no focal lesions
- ☐ Patent and homogeneously enhancing portal vein.
- ☐ No intra or extra hepatic biliary radicles dilatation.
- ☐ Bulky left suprarenal gland .

Patient's Name:

Fatma abd el salam gooda

Date:

2026-04-07

Referred by:

Prof.Dr. CT CAP+C

Hospital:

Goustave Roussey International Egypt

- ☐ Normal CT appearance of the pancreas, both kidneys, aorta and IVC.
- ☐ Normal CT appearance of the spleen .
- ☐ Regular filling of the urinary bladder showing no stones, masses .
- ☐ Unremarkable other pelvic organs.
- ☐ No retro-cranal, para-aortic or pelvic lymphadenopathy.
- ☐ No ascites.
- ☐ No worrisome lytic or sclerotic lesions.
- ☐ No vertebral structural collapse .

OPINION:

- " Left lower lung lobe area of consolidation intermingled with areas of ground glass attenuation likely infectious for clinical correlation . .

Dr.Reham Khedr, MD

Dr.Rahma Khader

Dr.Rahma Khader